

GUIDELINES

2016 European Guideline for the Management of Vulval Conditions

Abstract

Vulval conditions may present to a variety of clinicians, such as dermatologists, gynaecologists, and general practitioners. Women with these conditions are best managed by a multidisciplinary approach, which includes clear referral pathways between disciplines or access to a specialist multidisciplinary vulval service. Informed consent is a prerequisite for all examinations, investigations, and treatments. Consent is particularly important for intimate examinations of the anogenital area, and a chaperone should be offered in all cases. All efforts should be made to maintain a patient's dignity. Depending on symptoms and risk factors, screening for sexually transmitted infections (STI) should be considered. If the patient presents with vulval itch, particularly if also complaining of increased vaginal discharge, vulvovaginal candidiasis should be excluded. Sexual dysfunction should be considered in all patients with vulval complaints, either as the cause of the symptoms or secondary to symptoms, and assessed if appropriate. This guideline covers diagnosis and treatment of the more common vulval conditions frequently encountered in vulval clinics: vulval dermatitis (eczema), psoriasis, lichen simplex chronicus, lichen sclerosus, lichen planus, vulvodynia, and vulval intraepithelial neoplasia (VIN).

Scope

This guideline covers the more common conditions affecting the vulva:

1. Vulval dermatitis (eczema)
2. Psoriasis
3. Lichen simplex chronicus

4. Lichen sclerosus
5. Lichen planus
6. Vulvodynia
7. Vulval intraepithelial neoplasia (VIN)

General Advice for Delivery of Vulval Care

Vulval conditions may present to a variety of clinicians, including dermatologists, genitourinary medicine physicians, gynaecologists, and primary care physicians or general practitioners (GPs). Investigations and management span across these specialties, so women with vulval conditions are best managed through a multidisciplinary approach. This includes clear referral pathways between disciplines or access to a specialist multidisciplinary vulval service. Access to clinicopathological services is essential to allow discussion and review of histology results.

Physical Examination of the Patient

Informed consent is required for all examinations, investigations, and treatments. It is especially important for intimate examinations of the anogenital area, and a chaperone should be offered in all cases. This offer must be clearly documented in the patient's records. The proposed examination should be explained adequately before the patient undresses. Every effort should be made to maintain the patient's dignity, including providing privacy for dressing and undressing and ensuring adequate coverage during the examination.

Appropriate facilities and equipment for investigations should be available before beginning the examination. The examination room should be well lit, private, and soundproofed, with a suitable examination couch of adjustable height.

Dermatoses and sexually transmitted infections (STIs) may coexist. A woman with a pre-existing dermatosis may also acquire an STI. Therefore, STI screening should be considered based on symptoms and risk factors. In patients presenting with vulval itch—especially if accompanied by increased vaginal

discharge—vulvovaginal candidiasis should be excluded. If symptoms persist despite antifungal treatment, particularly when cultures are negative for *Candida*, a full genital examination should be performed and alternative diagnoses considered. These may include lichen sclerosus, lichen planus, lichen simplex chronicus, psoriasis, or neoplastic conditions—particularly human papillomavirus (HPV)-related vulval intraepithelial neoplasia (VIN) in younger women.

Sexual dysfunction should be evaluated in all patients with vulval complaints, as it may either be the underlying cause or a consequence of the condition, and should be assessed when appropriate.

Cutaneous disorders may be the initial manifestation of HIV-related immunosuppression. Skin diseases in HIV-positive individuals are often more severe and may present with atypical features. Signs suggestive of immunosuppression include numerous hyperkeratotic warts, treatment-resistant seborrhoeic dermatitis, and new-onset or severe psoriasis. HIV testing should be considered in all patients, particularly those with such presentations.

STI testing should also be specifically considered when genital ulcers are present—even if a known ulcerating dermatosis is present. In these cases, testing for herpes simplex virus and syphilis is recommended.